Case study: Kai

Condition: **Cerebral palsy – left spastic hemiplegia (GMFCS I, MACS II)**

9-year-old boy · Outpatient paediatric occupational therapy service

Kai Watson is a 9-year-old boy with **left spastic hemiplegic cerebral palsy** who lives with his parents, Rachel and Ben, in Townsville. He is in Year 4 at his local state primary school. Kai is classified at GMFCS level I and MACS level II; he walks and runs independently, but he uses his left hand mainly as a helper and often leaves it out of two-handed tasks. His therapy is funded through his NDIS plan.

Kai is enrolled in a three-week modified constraint-induced movement therapy programme at an outpatient paediatric occupational therapy service. Five days ago, a lightweight cast was applied to his right forearm and hand so that he would practise using his left hand during daily sessions and a home programme. The note from the day the cast was applied records that it was "well fitted" and that his fingers were warm and pink with normal sensation. His home diary shows that every practice session has been completed. Previous session notes describe him as cheerful and hard-working, and say that he rarely complains.

You have been asked to run this morning's session, which focuses on Kai's own goals of building Lego models and learning to tie his shoelaces. Kai bounces into the room and tells you about school camp next week, where he wants to "do the flying fox with both hands". Rachel props her phone on the shelf to film the session, explaining that she needs "a good video for Grandma and for the NDIS report". Ben asks whether Kai could do more hours each day. "If it's working, why not go harder?" he says.

During the first Lego activity, Kai keeps lifting his casted right arm and shaking it. He tells you his fingers feel "fizzy, like pins and needles" and that he "couldn't sleep last night because it was hurting". The fingertips showing at the end of the cast look puffy and pale, and his nail beds have a dusky, bluish tinge. They feel noticeably cooler than the fingers on his left hand. When you gently press one of his fingernails, the colour is slow to return. Kai can wriggle his fingers, but only slowly, and he says they feel "numb-ish". He winces when you gently straighten them.

When you mention what you have noticed, Ben shrugs. "The edge was rubbing his thumb, so I trimmed some of the padding out with scissors last night and taped it back down," he says. "He's fine. He's just trying to get out of the work." Rachel laughs and adds, "He's a bit of a drama king when he's tired." You notice several layers of brown packing tape wound firmly around the wrist end of the cast. Kai looks down at the table and goes quiet.

There is no clear documentation regarding whether the family were given written cast-care instructions, what time Kai's symptoms started, whether he took any pain relief overnight, or whether the cast has been checked since the day it was applied.

The receptionist knocks to say that the next family has arrived early and the waiting room is getting full. Rachel picks up her phone and asks whether you could "just get a quick clip of him doing the Lego first", because they need to drop Kai at school by ten. Kai picks up a Lego brick with his left hand and says, "Can we just do the Lego? I want to show Grandma."

You are the occupational therapy student running Kai's therapy session and your clinical supervisor is not currently in the room.

Present Kai's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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